

Infection Control Guidelines

Standard Precautions and Outbreak Management

MediAssist Health Network

Hospital Infection Control Committee

Document ref: NURS-IC-009 · Version 4.2

Access: Nurses, Doctors, Admin



These guidelines define the standard precautions and outbreak-management procedures that apply across all MediAssist facilities. Adherence is mandatory for all clinical staff and is monitored by the Infection Control team.

1. Five Moments of Hand Hygiene

The WHO 'Five Moments' framework is the foundation of infection prevention. Perform hand hygiene at each of these moments:

- 1 Before patient contact** — to protect the patient from your hands.
- 2 Before an aseptic procedure** — to protect the patient from infection.
- 3 After body-fluid exposure risk** — to protect yourself and the environment.
- 4 After patient contact** — to protect yourself and the environment.
- 5 After contact with patient surroundings** — to protect yourself and the environment.

Technique

Duration: alcohol-based hand rub **20–30 seconds**; soap-and-water handwash **40–60 seconds**. Wash with soap and water when hands are visibly soiled or after caring for a patient with *C. difficile*.

2. PPE Selection Guide

Scenario	Gloves	Apron/Gown	Mask	Eye Protection
Routine patient contact	Yes	No	No	No
Contact with blood/body fluids	Yes	Yes	Surgical	If splash risk
Suspected respiratory infection	Yes	Yes	N95	Goggles/shield
Aerosol-generating procedure	Yes	Full gown	N95/PAPR	Full face shield
Isolation room (contact)	Yes	Yes	Surgical	As indicated

Aerosol-generating procedures include intubation, extubation, bronchoscopy, open suctioning and non-invasive ventilation.

3. Standard Precautions

Standard precautions apply to **all patients regardless of diagnosis**. They treat blood and all body fluids, secretions and excretions (except sweat), non-intact skin, and mucous membranes as potentially infectious. They include hand hygiene, appropriate PPE, safe injection practice, and safe handling of sharps and linen.

4. Transmission-Based Precautions

Type	Example Organisms	Key Measures
Contact	MRSA, VRE, C. difficile, norovirus	Single room preferred; dedicated equipment; visitors gown + gloves
Droplet	Influenza, pertussis, meningococcus	Surgical mask within 1 metre; single room preferred
Airborne	TB, measles, chickenpox	Negative-pressure room; N95; limit patient transport

5. Healthcare-Associated Infection (HAI) Surveillance

The four HAIs under active surveillance are:

- **CAUTI** — catheter-associated urinary tract infection.
- **CLABSI** — central line-associated bloodstream infection.
- **VAP** — ventilator-associated pneumonia.
- **SSI** — surgical site infection.
- Any suspected HAI must be reported to the **infection control nurse within 24 hours**.
- A root-cause analysis (RCA) is conducted for every confirmed HAI, with learning shared at the monthly infection control committee.

6. Outbreak Management Protocol

An **outbreak** is defined as **two or more linked cases within 72 hours**.

Notification chain

Ward in-charge → Infection Control team → Chief Medical Officer (CMO).

Control measures

- Cohort affected patients; dedicate staff and equipment where possible.
- Implement visitor restrictions.
- Enhance environmental cleaning frequency and audit.
- Issue a staff-briefing communication using the standard template.

7. Waste Segregation

Bin Colour	Waste Type	Examples
Yellow	Biomedical / infectious	Dressings, blood-soaked items, body fluids
Red	Anatomical / contaminated plastics	Tubing, catheters, IV sets
Blue / White (translucent)	Glassware & sharps	Vials, ampoules, needles (puncture-proof)
Black	General (non-hazardous)	Office and domestic waste
White (cytotoxic)	Cytotoxic / pharmaceutical	Chemotherapy waste

Practice

Segregate at the point of generation. Biomedical waste bins are removed at least once per shift, and more frequently when $\frac{3}{4}$ full. Document collection in the waste register.

8. Needlestick & Sharps Injury Management

A sharps injury is a medical emergency for the exposed worker and must be acted on immediately.

- 1 Encourage the wound to bleed gently; wash with soap and running water (do not scrub).
- 2 Do not suck the wound; for splashes to eyes/mouth, irrigate with water or saline.
- 3 Report to the shift in-charge and attend the Emergency department / occupational health **without delay**.
- 4 Identify the source patient; risk-assess for HIV, Hepatitis B and Hepatitis C.
- 5 Commence post-exposure prophylaxis (PEP) if indicated — ideally within 2 hours, and no later than 72 hours for HIV exposure.
- 6 Complete the incident report and arrange baseline and follow-up serology.

Prevention

Never recap needles. Dispose of sharps at the point of use in the puncture-proof sharps container, filled no more than three-quarters.

9. Antimicrobial Stewardship (nursing role)

- Administer antimicrobials on time; delays compromise efficacy and resistance control.
- Prompt review of IV-to-oral switch when the patient is improving and afebrile.
- Ensure cultures are sent **before** the first antibiotic dose where feasible.
- Flag to the medical team any antibiotic continued beyond review date or without a documented indication.

10. Isolation Signage

Colour-coded door signs tell staff and visitors which precautions are in force before they enter:

- **Contact (yellow sign)** — contact precautions in force; gown and gloves before entry.
- **Droplet (blue sign)** — droplet precautions in force; surgical mask within 1 metre.
- **Airborne (red sign)** — airborne precautions in force; N95 plus a negative-pressure room.

Document ref: NURS-IC-009 · Version 4.2 · Owner: Hospital Infection Control Committee